MAINSPRING
HEALTHCARE PARTNERS

CAPABILITIES STATEMENT · TEMPORARY MANAGEMENT, RECEIVERSHIP AND STATE-DIRECTED OVERSIGHT

When the state has to step in, we are the operators who step in.

Licensed nursing home administrators and nurse leaders who take over a distressed nursing facility or assisted living residence on short notice, make it safe, correct what the survey found, put the finances under control, and hand it back — with reporting the Department can rely on from the first week.

Days	72 hrs	Weekly	Fixed
Licensed administrator and DON in the building	Resident-safety assessment and written stabilization plan	Reporting in the Department's or the court's format	Pre-agreed fees that fit a state contract or order

PREPARED FOR

Colorado Department of Public Health and Environment
Health Facilities and Emergency Medical Services Division
Acute and Nursing Facilities Branch

PREPARED BY

Mainspring Healthcare Partners
Felix Strickland III, Managing Director, Operations and Compliance
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FOR THE DEPARTMENT, THE ATTORNEY GENERAL AND THE COURTS

Every remedy Colorado can impose on a failing facility needs someone qualified to run the building while it is imposed.

Colorado counts a receiver or temporary manager, a directed plan of correction, and intermediate conditions such as a required consultant or Department monitoring among its enforcement remedies. We are built to be that temporary manager, receiver, consultant or monitor — and operator of record when an order calls for one — for skilled nursing and nursing care facilities, assisted living residences including secured memory care, and continuing care communities.

WHERE WE ARE ENGAGED

- **Temporary management** imposed as an enforcement remedy, state or federal, where the Department needs a qualified administrator in the building immediately and reporting it can rely on
- **Receivership** under C.R.S. 25-3-108 — court-appointed operation of a long-term care facility through to lease, sale or orderly closure, led by a licensed nursing home administrator as the statute requires
- **Independent monitoring and consultation** under a directed plan of correction, conditional licence, settlement or consent order, with verification the Department does not have to take on faith
- **Managed closure and resident relocation** when a licence is surrendered, suspended or revoked — safe transfers, family communication, records and resident trust funds accounted for
- **Rapid stabilization** after immediate-jeopardy findings or an abandonment event, where the priority is resident safety tonight
- **Forensic accounting** of a facility's funds, payroll and related-party transactions for the Department, the receiver or the Attorney General

WHAT THE DEPARTMENT GETS

- A licensed administrator and a director of nursing in the building within days, matched to Colorado licensure and the facility's survey history
- A first-72-hour resident-safety assessment and a written stabilization plan
- Weekly reporting in the format the Department or the court specifies: census, staffing, clinical risk, cash, vendor status, open deficiencies
- Financial control from day one — cash, payroll, resident trust funds, accounts payable — with an independent accounting at the end
- A defined exit: transfer to a qualified operator, a sale, or an orderly closure with every resident placed
- Fixed, pre-agreed fees that fit a state contract, a court order or an enforcement remedy

How this fits Colorado's framework

Enforcement activity under C.R.S. 25-1.5-103 includes civil money penalties, conditional licensure, suspension or revocation, appointment of a receiver or temporary manager, a directed plan of correction, and intermediate conditions such as a required consultant, Department monitoring or additional training. **Receivership** under C.R.S. 25-3-108: when a licence is revoked, not renewed or summarily suspended, the Department, licensee or owner may ask the district court to appoint a receiver — who must be a licensed nursing home administrator, operates under Department standards, and serves up to 180 days unless extended — so residents are not moved before a lease, sale or closure is arranged. **Federal temporary management** under 42 C.F.R. 488.415 remains available for certified facilities as an alternative to termination.

Summarized for orientation only; the governing text is the current Colorado Revised Statutes, 6 CCR 1011-1 and 42 C.F.R. Part 488. We work within the specific order, contract or remedy that governs each engagement.

OPERATORS, NOT CONSULTANTS

A mandated engagement is only as good as the people in the building at 2 a.m.

Our principals and network are working administrators and nurse leaders — people who hold the licence, have run buildings through survey, and have taken over failing operations before. That is the difference between a report that describes the problem and a building that is safe tonight.

1

Day 0–3

Administrator and DON in the building. Resident-safety assessment. Financial controls taken over. First report to the Department.

STABILIZE

2

Weeks 1–6

Deficiencies corrected with evidence. Staffing rebuilt. Vendors and payroll current. Weekly reporting begins and does not stop.

CORRECT

3

Months 2–6

Systems that hold without us. Census and cash stabilized. Options prepared for the Department: transfer, sale or closure.

REBUILD

4

Exit

Hand-off to a qualified operator, or an orderly closure with every resident placed and every dollar accounted for.

RESOLVE

RESULTS FROM OUR LEADERSHIP'S OPERATING RECORD

CONTINUING CARE RETIREMENT COMMUNITY · MICHIGAN

Back to budget, off agency staffing, through survey, and sold.

Staffing recalibrated to budget, department heads managing paid hours per resident day daily, agency use eliminated — while completing the annual survey.

$40k/mo
Labor savings, sustained eight months

$320k
Cumulative savings over the engagement

0
Agency staffing by the end

Sold
Owner completed the sale on a stable operation

SKILLED NURSING & REHABILITATION · TENNESSEE

Rehab census more than doubled. Survey deficiency-free.

Regulatory gaps ranked and assigned to department leaders; short-term rehab rebuilt around Medicare Part A and managed care; survey readiness run daily.

20–25 → 45–55
Short-term rehab average daily census

50+
Admissions a month

$250k/mo
Added revenue at the higher operating level

0
Deficiencies, October 2019 annual health survey

These results reflect the individual operating experience of Mainspring's leadership before the firm was formed, not Mainspring corporate engagements. Figures are as reported by the leader responsible for the period; survey results as recorded by the state survey agency. Facilities are not named; references are available to the Department on request.

Tech-driven, by habit

We run our own operating businesses on the latest AI computing, and every mandated engagement gets the same stack: a daily census, staffing and cash brief · payroll and related-party anomaly monitoring from day one · a deficiency tracker with evidence attached to every correction · a documentary record the Department or the court can audit.

LEADERSHIP

Who the Department would actually be working with.

FS

Felix Strickland III

Managing Director, Operations and Compliance · Licensed Nursing Home Administrator

Healthcare executive with more than sixteen years of leadership across skilled nursing, assisted living, memory care and senior living — as Administrator, Executive Director, Chief Executive Officer, Director of Operations and consultant, with multi-state experience. His focus is operational performance, financial improvement, census growth, regulatory compliance and quality outcomes; hands-on, data-driven, and uncompromising on resident-centred care. Bachelor's degree in Management and Project Management.

Licensed NHA · Multi-state · SNF · AL · memory care · Survey & plan of correction · Turnaround

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DB

David Bloch

Managing Partner

More than twenty years of executive experience with a focus on operational efficiency across technology, real estate and healthcare. Began with Fortune 500 companies and management consulting, including IBM and Verizon; moved into private equity and structured debt, then ownership of real estate portfolios; now applies that combined corporate, financial and ownership expertise to healthcare operations in a highly regulated industry. Known for rapidly assessing complex organizations and delivering lasting, measurable improvement.

20+ years executive · IBM · Verizon · Private equity & structured debt · Financial control · Asset management

db@mainspringhcp.com · 650-418-2280

Contracting and the network behind the principals

We contract directly with state agencies, courts and appointed receivers, and can serve as operator of record under a temporary-management or receivership order. Scope, deliverables, reporting cadence and fixed fees are set out in writing before work begins. Behind the principals is a vetted network of licensed administrators, directors of nursing and department leaders, so a Colorado engagement is staffed by people matched to Colorado's regulations. Statements of qualifications, references and certificates of insurance are available on request.

Tell us about the facility.

Same-day response. In one conversation we will say whether we can have leadership in the building this week, and what it would involve.

ENGAGEMENT LEAD
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Mainspring Healthcare Partners provides operational, financial and advisory services and is not a law firm, accounting firm or government agency. Regulatory outcomes depend on the facts of each facility and the decisions of the survey agency and the courts; no outcome is guaranteed. Mainspring is an equal opportunity employer and does not discriminate in employment, engagement or the delivery of services.